PointClickCare®

Centers for Medicare & Medicaid Services
Department of Health and Human Services
Attention: CMS-0042-NC
7500 Security Boulevard
Baltimore, MD 21244

Re: Response to RFI on Health Technology Ecosystem (CMS-0042-NC)

Dear CMS and ASTP/ONC Representatives,

On behalf of PointClickCare, a leading technology vendor serving the post-acute and long-term care (LTPAC) sector, we appreciate the opportunity to respond to the Request for Information (RFI) on the Health Technology Ecosystem.

More than 27,000 LTPAC provider organizations, 3,600 ambulatory clinics, 2,800 hospitals, 350 risk-bearing providers, 70 state and government agencies, and every major U.S. health plan trust us to help them navigate a changing healthcare landscape. Our marketplace of 400+ integrated partners helps organizations customize their suite of solutions to connect care, billing, and operations — driving efficiency and better outcomes.

We commend CMS and ASTP/ONC for your commitment to advancing interoperability, patient access, and innovation in digital health, and we strongly support efforts to align technology policies with the needs of underserved care settings, including skilled nursing facilities (SNFs), assisted living facilities, and other healthcare stakeholders.

As a technology organization deeply embedded in workflows across the healthcare ecosystem, we offer the following insights to questions under the Patient and Caregivers, Technology Vendor, and Value-based Care Organization sections. Our feedback focuses on actionable recommendations to:

1. **Patient and Caregiver** information replies regarding access to and use of technology and APIs (PC-5, 9, and 11)
2. **Stimulate developer interest** in LTPAC-specific digital health solutions (TD1 and 2).
3. **Standardize identity and APIs** to reduce fragmentation (TD-3, 4, and 9).
4. **Optimize USCDI and certification** for post-acute care workflows (TD-7 and10).
5. **Leverage TEFCA** as the central framework for data exchange (TD-5, 6, and16).
6. **Value-based Care Organizations** advancing digital health adoption and compliance (VB -1, 4, 7, and13)

Our recommendations emphasize pragmatic steps to reduce provider burden, ensure scalability, and address unique challenges across the digital health ecosystem. We welcome further collaboration to demonstrate real-world use cases and pilot solutions.

Sincerely,

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Patient and Caregivers

PC-5. What can CMS and its partners do to encourage patient and caregiver interest in these digital health products?

b. What changes would enable timely access to high quality CMS and provider generated data on patients?

To encourage patient and caregiver engagement with digital health products, CMS and its partners should prioritize timely access to high-quality data and targeted support for underserved care settings. We encourage CMS to consider the following:

1. Enabling Data Access for Patients and Caregivers

- *Expand Federal Funding for Interoperability Efforts: Post-acute and long-term care (LTC) settings have been historically excluded from interoperability funding. CMS should allocate dedicated resources to integrate these providers into nationally trusted exchanges ensuring seamless data flow across skilled nursing facilities (SNF).*
- *Standardize APIs for Real-Time Data Sharing: CMS should accelerate adoption of standardized Admission/Discharge/Transfer (ADT) and Continuity of Care Document (CCD) APIs, enabling caregivers—especially those acting as proxies—to monitor gaps in care and intervene proactively.*
- *Prioritize Proxy Access: Medicare beneficiaries in LTC settings often rely on caregivers to manage their care. CMS should formalize proxy-access protocols in digital tools to empower caregivers with unified data views.*
- *Integrate LTC Pharmacies into Data Exchange: Long-term care (LTC) pharmacies play a critical role in care coordination, particularly for residents in skilled nursing facilities (SNFs) and assisted living facilities (ALFs). These pharmacies often deploy consultant pharmacists and clinical staff who conduct medication regimen reviews, reconcile complex medication profiles, and support transitions of care — including when residents return to the community. Despite their enhanced clinical and logistical roles compared to traditional retail pharmacies, LTC pharmacies are typically reimbursed under the same Part D structure, without recognition of their expanded responsibilities.*
    - *CMS, in cooperation with other applicable federal agencies, should encourage greater industry collaboration in refining the nuanced roles and workflows of these stakeholders across the ecosystem. Furthermore, CMS should explore a supplemental dispense fee, or a per-member payment (VBC-aligned) incentive to sustain and scale these services.*

c. What specific opportunities and challenges exist to improve accessibility, interoperability and integration of clinical data from different sources to enable more meaningful clinical research and generation of actionable evidence?

To drive interest and trust in digital health tools among Medicare beneficiaries—especially in underserved settings like SNFs and ALFs—CMS and partners should:

- *Develop a Voluntary Certification Program*

    - *While CMS should not directly approve products, it could partner with ONC to create a "CMS Trustmark" certification for digital health tools that meet efficacy, usability, and interoperability standards (e.g., HL7 FHIR, HIPAA compliance).*
- *Align Reimbursement with Digital Health Use*
    - *Introduce supplemental payments (e.g., via Medicare Advantage or ACOs) for providers using certified tools (e.g., remote monitoring in SNFs).*
    - *Test value-based payment models for LTC pharmacies (e.g., per-member care coordination fees) to incentivize adoption of medication management apps.*
- *Targeted Patient/Caregiver Outreach*
    - *Launch Medicare.gov resources and SHIP counselor trainings to educate beneficiaries on accessing and using digital health tools.*
    - *Partner with advocacy groups to highlight tools for proxy access and caregiver support.*
- *Reduce Barriers in LTC Settings*
    - *Fund Wi-Fi/infrastructure upgrades in SNFs/ALFs to support digital tool use.*
    - *Require usability testing for certified tools to ensure accessibility for older adults and caregivers.*

PC-9. Given that the Blue Button 2.0 API only includes basic patient demographic, Medicare coverage, and claims data (Part A, B, D), what additional CMS data sources do developers view as most valuable for inclusion in the API to enable more useful digital products for patients and caretakers?

c. Is there non-CMS data that should be included in the API?

To maximize the utility of digital health products for patients and caregivers, the Blue Button 2.0 API should integrate key non-CMS data sources that fill critical gaps in care coordination, coverage transparency, and risk management. While claims data provide a foundational view of a patient's healthcare utilization, developers and value-based care (VBC) organizations need additional real-world insights to build actionable tools. Below are the most valuable non-CMS data categories for inclusion:

1. Historical Claims and Coverage Details

Prior authorization records, coverage limits, and benefit utilization summaries (e.g., percentage of inpatient days covered, remaining SNF benefit days) would significantly enhance patient and caregiver decision-making. For example:

- *A caregiver managing a Medicare beneficiary's post-acute care could avoid coverage surprises by seeing real-time updates on remaining SNF or home health eligibility.*
- *Digital health apps could alert patients when they approach therapy visit caps or prior authorization expirations, reducing disruptions in care.*

2. Encounter-Level Clinical Data from Providers

SNF therapy logs, medication adherence records, and care plan milestones are essential for VBC organizations and caregivers managing chronic conditions. While CMS does not generate this data directly, requiring and incentivizing providers to share it via APIs would enable:

- *Accurate risk stratification (e.g., tracking missed dialysis sessions or rehab visits to predict hospitalization risks).*
- *Personalized care coordination tools (e.g., apps that reconcile medication lists with actual pharmacy fills or flag lapses in home health visits).*

3. Pharmacy and Prescription Data

Despite being a critical touchpoint, LTC pharmacy records (e.g., medication reconciliation reports, adherence trends) are often siloed outside CMS systems. Integrating this data would:

- *Help caregivers and clinicians prevent medication errors during transitions (e.g., from SNF to home).*
- *Support closed-loop referrals between prescribers and pharmacists in VBC arrangements.*

By layering non-CMS data—particularly real-time encounter records, pharmacy data, and SDOH—into Blue Button 2.0, CMS can transform the API from a claims repository into a dynamic care coordination engine. This aligns with the emphasis on VBC-ready analytics, ultimately empowering patients and caregivers with a complete, actionable view of care.

PC-11. How are health information exchanges (HIEs) currently helping to advance patient access to health information in the real world?

a. How valuable, available, and accurate do you find the data they share to be?

Health Information Exchanges (HIEs) play a pivotal but still-evolving role in advancing patient access to health information. While they hold valuable clinical data that can empower patients, their current focus remains largely on provider, payer, and public health use cases rather than direct patient engagement. Below is an assessment of their real-world impact, based on the feedback provided:

1. Value of HIE Data

HIEs aggregate critical clinical information—such as lab results, medication histories, and care summaries—that can significantly enhance care coordination and patient decision-making. This data is meaningful in theory, but its practical utility for patients depends on:

- *Interoperability standards (e.g., whether HIEs support FHIR APIs for patient-facing apps).*

- *Inclusion of patient-centric data (e.g., visit notes, discharge instructions) beyond traditional provider-to-provider exchanges.*

2. Availability Challenges

The accessibility of HIE data is highly inconsistent, varying by:

- *Geographic Participation: Rural and underserved areas often lack robust HIE coverage, leaving gaps in longitudinal records.*
- *Provider Engagement: Hospitals and clinics may contribute data unevenly, with some opting out due to cost or competitive concerns.*
- *Patient Access Points: Few HIEs offer direct patient portals or APIs for third-party apps (e.g., Apple Health, caregiver platforms).*

3. Accuracy and Reliability

Data quality in HIEs is mixed, influenced by:

- *System Heterogeneity: HIEs often integrate data from providers with varying levels of technology sophistication, including legacy systems that may lack interoperability standards. This can introduce inconsistencies in data formatting, completeness, or timeliness, impacting overall accuracy and reliability.*
- *Normalization Efforts: Variations in how providers code and submit data (e.g., conflicting medication lists) can lead to inconsistencies.*
- *Maintenance Practices: HIEs with strong governance and data reconciliation processes tend to offer higher accuracy.*

Technology Vendors

1. Ecosystem

TD-1. What short term (in the next 2 years) and longer-term steps can CMS take to stimulate developer interest in building digital health products for Medicare beneficiaries and caregivers?

To accelerate digital health innovation for Medicare beneficiaries and caregivers, CMS should take immediate action to lower barriers to entry and align incentives with market needs. Targeted subsidies or rates could spur developer investment in high-priority gaps—such as caregiver coordination tools or SNF-specific interoperability—by de-risking early-stage development. CMS should also expand FHIR adoption in post-acute care by requiring FHIR-based APIs in CMS-funded programs (e.g., MDS) and funding specialized workgroups to address LTPAC use cases (e.g., medication reconciliation). These steps would create immediate demand signals while laying the groundwork for scalable, interoperable tools.

TD-2. Regarding CMS Data, to stimulate developer interest:

a. What additional data would be most valuable if made available through CMS APIs?

The availability of beneficiary attribution data through CMS APIs would significantly enhance innovation in value-based care (VBC) by addressing a critical information gap. Currently, this data is only accessible to risk-bearing entities, creating barriers for developers creating solutions to:

- *Improve care coordination by enabling real-time identification of a provider's attributed patient population*
- *Develop more accurate risk stratification tools*
- *Create patient engagement solutions tailored to specific VBC models*
- *Enhance transparency in performance measurement and quality reporting*

Making beneficiary attribution data available through APIs - with appropriate privacy safeguards - would particularly benefit:

- *Care management platform developers*
- *Population health analytics tools*
- *Patient engagement solution providers*
- *Quality measurement and reporting systems*

The data would be most valuable if it included:

- *Timely (preferably near real-time) attribution updates*
- *Historical attribution trends*
- *Model-specific attribution details (e.g., ACO, MA, bundled payments)*
- *Associated risk scores or other relevant quality metrics*

This enhancement would stimulate developer interest by enabling creation of more targeted, effective solutions that directly support CMS's value-based care objectives while maintaining appropriate data privacy protections.

d. What other APIs should CMS and ASTP/ONC consider including in program policies to unleash innovation and support patients and providers?

To empower developers and address critical gaps in care coordination, CMS should prioritize API access to:

1. *Post-Acute & LTPAC Data (MDS/iQIES)*
    - *This would enable seamless care transitions by exposing standardized patient assessments, quality measures, and functional status data currently locked in siloed systems.*
2. *Standardized Laboratory Data (LOINC/SNOMED-Coded)*
    - *Why it matters: Replaces fragmented, vendor-specific lab compendiums with CLIA-aligned structured data - critical for antimicrobial stewardship, chronic disease management, and public health surveillance.*

- *This would finally enable "test once, result everywhere" workflows across EHRs, HIEs, and patient apps for objective clinical data that already has vocabulary concepts and standards and only lacks laboratory requirements.*

3. *Value-Based Care Attribution Data*
    - *As previously noted, this would unlock transformative applications for risk stratification, care gap identification, and model-specific performance analytics.*

2. Digital Identity

TD-3. Regarding digital identity implementation:

a. What are the challenges and benefits?

Effective digital identity management presents both opportunities and complexities that warrant careful evaluation.

Primary Challenges

1. *Non-Standard Implementations*
    - *Custom authentication systems introduce security vulnerabilities, interoperability issues, and long-term maintenance burdens.*
    - *Example: State-specific implementations, already underway and/or consideration, may conflict with federal identity frameworks, increasing costs and fragmentation.*
2. *Legacy System Limitations*
    - *Aging infrastructure often lacks support for modern protocols (e.g., FIDO2, OAuth 2.0), complicating secure integration.*
3. *Regulatory Variability*
    - *Differing state and federal requirements create compliance hurdles and delay adoption.*

Recommended Approaches

1. *Adopt Established Standards*
    - *Implement widely accepted protocols (e.g., SAML, OpenID Connect, NIST 800-63B) to ensure security and interoperability.*
2. *Align with Federal Frameworks*
    - *Leverage federal solutions (e.g., Login.gov, GSA's Identity Proofing Guidelines) to reduce redundant development efforts.*
3. *Prioritize Incremental Modernization*
    - *Phase legacy system upgrades to balance security improvements with operational continuity.*

We believe that standardization mitigates risk, reduces costs, and ensures scalable identity solutions. We recommend a collaborative approach to align state and federal initiatives where feasible.

c. What impact would mandatory use of the OpenID Connect identity protocol have?

Mandating OpenID Connect (OIDC) as the exclusive identity protocol would present substantial operational and strategic challenges without proper infrastructure in place. Currently, the OIDC ecosystem relies heavily on a limited set of commercial providers, creating inherent risks around service continuity, data privacy, and equitable access. The absence of a federally operated OIDC provider raises concerns about transparency, accountability, and long-term sustainability.

In healthcare specifically, this approach could introduce compliance complexities with HIPAA and other regulatory frameworks, as third-party identity providers may not consistently meet stringent healthcare security and privacy requirements. Dependency on external commercial platforms also poses business continuity risks should providers alter service terms or discontinue offerings.

A more viable path forward would involve developing federal OIDC capabilities aligned with healthcare needs, potentially through integration with existing government identity solutions. This would mitigate vendor lock-in while ensuring the necessary safeguards for sensitive health data.

3. Technical Standards and Certification

TD-4. How can CMS better encourage use of open, standards-based, publicly available APIs over proprietary APIs?

To achieve meaningful progress, we urge a focus on standards-based APIs, where CMS should adopt a supportive, ecosystem-driven approach that aligns stakeholder incentives with interoperability goals.

First, ONC should prioritize collaborative certification—working with vendors to adopt standards like FHIR through technical assistance and public recognition of compliant solutions.

Second, federal standards should also be acknowledged with the same parity and/or in conjunction with state specific laws to eliminate fragmentation. A unified national framework will reduce burdens on developers and accelerate implementation.

Third, funding mechanisms—particularly federal Medicaid dollars for HIEs and state initiatives—should be tied to compliance with nationally recognized standards. Reimbursement policies can further drive adoption; for example, CMS could adjust payment rates to favor providers who leverage certified APIs for data exchange and demonstrate consistent utilization of such exchanges.

Fourth, interoperability efforts must address technical and regulatory gaps. Outdated standards (e.g., CCDA v1) should be deprecated in certification programs, and new standards must be developed with input from all care settings (LTPAC, labs, behavioral health) to ensure APIs reflect diverse needs. Collaboration with the FDA is also essential to mandate public API support for all approved health technologies (e.g., devices, apps) and government health datasets (e.g., SDoH).

Finally, to prevent subversion of standards, alternate data-sharing methods should be restricted unless no viable standard exists. Certification rules and exchange should explicitly prioritize modern, scalable interfaces over proprietary workarounds.

Together, these steps would create a system where adherence to standards is both incentivized and obligatory, with transparency ensuring public trust.

TD-5. How could a nationwide provider directory of FHIR endpoints improve access to health information for patients, providers, and payers? Who should publish such a directory, and should users bear a cost?

A nationwide FHIR endpoint directory could significantly improve health information access by enabling seamless, standards-based exchange for patients, providers, and payers. However, its design must address key technical, governance, and equity challenges to realize its potential. Below are our recommendations:

1. Value of a Unified Directory

- *For Patients: Simplifies access to their records across providers (e.g., via apps) by identifying where data is stored.*
- *For Providers: Reduces friction in care coordination (e.g., locating endpoints for referrals or public health reporting).*
- *For Payers: Supports prior authorization, claims adjudication, and quality measurement by automating data retrieval.*

This aligns with TEFCA's vision for FHIR-based exchange, but requires clarity on scope. A "provider directory" should map organizational endpoints (e.g., hospitals, clinics, labs)—not individual clinicians—since providers often work across multiple systems.

2. Governance & Transparency

- *Public, Non-Proprietary Access: To prevent favoritism or information blocking), the directory should be freely accessible and maintained by a trusted federal entity (e.g., ONC in partnership with TEFCA's RCE).*
- *Cost Structure: A nominal fee for endpoint registration could sustain operations but must not deter participation. Federal funding could offset costs for safety-net providers.*

3. Security & Authorization

- *Guardrails Against Misuse: The directory must integrate authentication protocols (e.g., OAuth 2.0) and enforce contractual agreements between entities to prevent unauthorized access.*
- *Third-Party Oversight: Regular audits of endpoint security practices (e.g., adherence to FHIR security IG) should be mandated.*

5. Implementation Suggestions

- *Leverage existing TEFCA Infrastructure: Build on existing QHIN frameworks to avoid duplication.*
- *Phase In Requirements: Start with mandatory endpoint registration for certified EHRs, then expand to other entities (e.g., payers, FDA-regulated devices).*
- *Pilot with High-Impact Use Cases: Focus on patient access (via apps) and payer-provider data exchange to prove utility.*

TD-6. What unique interoperability functions does TEFCA perform?

TEFCA, as the only federally endorsed national network, has served an important role of aligning stakeholders and states around a unified approach to advancing interoperability across the U.S. Recognizing this we would encourage CMS and HHS to lean into promoting TEFCA and/or other trusted networks and encourage states to align their efforts around TEFCA instead of creating conflicting frameworks. CMS, like IHS already has, should join TEFCA and leverage it to support CMS goals and programs (e.g. quality measurement, waste, fraud and abuse etc.). CMS should also consider incentivizing providers, payers, and Medicaid agencies to actively participate in TEFCA.

a. What existing alternatives should be considered?

Due to the significant variability in the U.S. healthcare system no single network or interoperability solution will fill all the needs of stakeholders. Recognizing this is essential to success. While we believe TEFCA has an important role to play in standardizing common national use cases there are other mechanisms for exchange that can provide value and be complimentary in the proper capacities. It is important to ensure these alternatives mechanism do not duplicate solutions happening at the national level.

b. Are there redundant standards, protocols or channels or both that should be consolidated?

Consolidation Opportunities:

- *A Standards Harmonization: Align overlapping specifications (e.g., CARIN Alliance IG overlaps with QHIN requirements)*
- *Channel Rationalization: Migrate state Medicaid data exchanges to TEFCA infrastructure*
- *Protocol Simplification: Deprecate legacy SOAP-based web services in favor of FHIR-based exchange*

Implementation Recommendations:

1. *Align Parallel Systems: Establish clear timelines for transitioning state HIEs to TEFCA participants*
2. *Adoption Incentives: Tie Medicare MIPS/APM participation to TEFCA connectivity by a specific future date.*
3. *Gap Remediation: Fund testing tools for specialty-specific data elements (e.g., behavioral health CCD extensions)*

TD-7. To what degree has USCDI improved interoperability and exchange and what are its limitations?

a. Does it contain the full extent of data elements you need?

b. If not, is it because of limitations in the definition of the USCDI format or the way it is utilized?

The US Core Data for Interoperability (USCDI) has significantly advanced nationwide health data exchange by establishing essential baseline consistency. Its standardized data elements serve as the foundational language for clinical documents (like CCDs) and system-to-system interfaces, delivering key benefits:

1. *Reduced Implementation Complexity*
    - *Provides codified elements that unify disparate exchange methodologies*
    - *Lowers maintenance burdens by replacing state/vendor-specific variations*
    - *Enables reliable care coordination across settings through structured data*

2. *Enabled Critical Use Cases*
    - *Supports patient access via standardized API payloads*
    - *Facilitates care collaboration in value-based arrangements*
    - *Forms the backbone for federal interoperability programs*

However, USCDI faces important limitations requiring attention:

1. *Version Lag Challenges*
    - *Slow adoption cycles for emerging needs (new lab tests, specialty data elements)*
    - *Creates "lowest common denominator" effect that can stifle innovation*
2. *Coverage Gaps*
    - *Excludes critical auxiliary mandates (i.e. labs, radiology centers)*
    - *Lacks specialty-specific data elements needed for comprehensive care*

Recommended Improvements:

- *Accelerate update cycles through stronger stakeholder collaboration*
- *Expand to include diagnostic/longitudinal data from all care settings*
- *Develop implementation guides for emerging use cases while maintaining core stability*

USCDI remains our most effective tool for baseline interoperability - but its full potential requires addressing these constraints through sustained public-private partnership.

c. If so, would adding more data elements to USCDI add value or create scoping challenges? How could such challenges be addressed?

Expanding the US Core Data for Interoperability (USCDI) with meaningful *rather than simply* more *data elements could significantly enhance interoperability while avoiding scope creep. For example, adding standardized functional status metrics—such as Section GG self-care and mobility scores—could streamline post-acute transitions and support better alignment across care settings. Similarly, structured advance care planning documents (e.g., POLST forms) would improve care transitions without requiring universal EHR adoption. To balance*

standardization with flexibility, USCDI could adopt a more modular approach, allowing different care settings (e.g., hospitals vs. SNFs) to implement the concepts and data subsets applicable to each while maintaining core alignment. This would ensure USCDI evolves purposefully rather than becoming unwieldy. The key is not to obsolete USCDI but to refine its scope—prioritizing high-impact data that drives care quality, not just volume.

d. Given improvements in language models, would you prefer a non-proprietary but less structured format that might improve data coverage even if it requires more processing by the receiver?

The emergence of sophisticated language models presents an opportunity to rethink rigid data standardization, particularly for domains where exhaustive codification has proven impractical. A non-proprietary, less structured approach—leveraging natural language processing (NLP) and semantic similarity—could indeed resolve long-standing interoperability challenges in areas like prescription sig instructions (SIG), where a decade of codification efforts (e.g., structured SIG standards) has seen limited adoption due to clinical variability and workflow friction.

For SIG data, where providers naturally express the same intent in multiple ways (e.g., "take 1 tablet twice daily" vs. "1 tab BID"), language models could interpret intent accurately without forcing artificial standardization. This flexibility would:

Reduce implementation barriers by accommodating real-world clinical language rather than demanding perfect adherence to coded values.

Preserve nuance in instructions that structured fields often strip away (e.g., conditional dosing like "take with food if upset stomach occurs").

Future-proof systems by allowing new phrasing variants to be understood without requiring updates to value sets.

However, this approach should be domain-specific. While unstructured or semi-structured formats are ideal for narrative-rich data (e.g., clinical notes, SIGs), critical discrete data (e.g., lab results, allergies) still benefit from standardization. The key is a hybrid strategy:

Use NLP for interpretation where variability is inherent (e.g., SIGs, care plan notes).

Retain structured standards for high-risk, discrete data elements.

Less structured formats, powered by modern language models, are not just viable but preferable for certain use cases—especially those where forced codification has historically failed. The goal should be interoperability of meaning, not just interoperability of syntax.

TD-8. What are the most effective certification criteria and standards under the ONC Health IT Certification Program?

The most effective ONC Health IT Certification criteria today are those that close critical interoperability gaps *while sunsetting redundant or low-value requirements. Key focus areas should include:*

1. *Interop-Centric Certification*
    - *Retain and strengthen standards for FHIR APIs, USCDI, and patient/provider data exchange, as these directly address systemic fragmentation.*
    - *Eliminate or streamline criteria that impose high-cost burdens without commensurate interop benefits (e.g., overly prescriptive functionality mandates).*

2. *Replace Advanced Technical Criteria with Modern Frameworks*
    - *Example: §170.315(b11) (Decision Support Interventions) could shift from rigid certification to self-attestation of a Responsible AI (RAI) framework, reducing vendor burden while ensuring ethical AI adoption.*

TD-9. Regarding certification of health IT:

a. What are the benefits of redefining certification to prioritize API-enabled capabilities over software functionality?

Redefining certification to prioritize API-enabled capabilities would streamline interoperability by ensuring standardized data access across systems. While APIs inherently rely on underlying software functionality, shifting focus to API performance—rather than prescriptive features—could free up vendor resources for innovation. Simply put, without standardized APIs, true interoperability is impossible. The key benefit is creating a future-proof framework where data fluidity, not rigid functionality, drives certification.

b. What would be the drawbacks?

Overemphasizing APIs risks neglecting foundational software functionality that enables those APIs to work. For example, APIs for lab results or patient portals depend on robust backend systems to capture and structure data. A purely API-driven approach might also inadvertently exclude legacy systems or smaller providers lacking resources to modernize. Balance is critical: APIs should augment, not replace, core functionality.

c. How could ASTP/ONC revise health IT certification criteria to require APIs to consistently support exchanging data from all aspects of the patient's chart (for example, faxed records, free text, discrete data)?

To ensure APIs exchange comprehensive patient data (faxes, free text, discrete fields), ONC should:

1. *Limit APIs to "sources of truth" to avoid duplicate/confusing data.*

2. *Use TEFCA governance to maintain fidelity, with optional unstructured data sharing (per TV) for use cases where NLP/deduplication tools exist.*
3. *Exclude low-value unstructured data unless standardized (e.g., USCDI).*

d. What policy changes could CMS make so providers are motivated to respond to API- based data requests with best possible coverage and quality of data?

CMS could:

- *Tie reimbursement to USCDI compliance, ensuring data quality.*
- *Mandate CEHRT for all CMS-funded clinicians, closing LTPAC gaps.*

e. How could EHRs capable of bulk data transfer be used to reduce the burden on providers for reporting quality performance data to CMS? What capabilities are needed to show benefit? What concerns are there with this approach?

Bulk APIs could reduce reporting burdens if:

- ***EHRs recoup development costs** via CMS funding.*
- ***Focus on population-level insights** rather than raw data dumps.*

We ask that CMS consider not creating an over-reliance on bulk transfers which might overlook nuanced care contexts for specific settings of care.

TD-10. For EHR and other developers subject to the ONC Health IT Certification Program, what further steps should ASTP/ONC consider to implement the 21st Century Cures Act's API condition of certification (42 U.S.C. 300jj-11(c)(5)(D)(iv)) that requires a developer's APIs to allow health information to be accessed, exchanged, and used without special effort, including providing access to all data elements of a patient's electronic health record to the extent permissible under applicable privacy laws?

ASTPONC should prioritize use-case-centric access (e.g., treatment, payment, and operations [TPO]) while safeguarding patient privacy and data stewardship. Key steps include:

1. *Standardize Consent and Data Privacy Rules*
   - *Adopt a unified national standard for consent and protected data status to avoid a patchwork of conflicting state-level requirements.*
2. *Enforce Transparent API Governance*
   - *Require certified APIs to log and disclose:* who *is accessing data,* what *is being requested, and* how *it will be used—ensuring accountability without stifling innovation.*
3. *Clarify EHR Vendor Protections*

- *Allow EHR vendors to set reasonable security and technical requirements for third-party apps (e.g., data localization, developer expertise) to prevent frivolous information-blocking claims .*

This approach balances unfettered access with patient control, ensuring APIs meet both interoperability goals and privacy obligations.

TD-11. As of January 1, 2024, many health IT developers with products certified through the ONC Health IT Certification Program are required to include the capability to perform an electronic health information export or "EHI export" for a single patient as well as for patient populations (45 CFR 170.315(b)(10)). These health IT developers are also required to publicly describe the format and syntax of the EHI export. Notably, how EHI export was accomplished was left entirely to the health IT developer. Now that this capability has been in production for over a year, CMS and ASTP/ONC seek input on the following:

a. Should this capability be revised to specify standardized API requirements for EHI export?

We recognize the importance of EHI export and do not believe that revision is appropriate here. Standardized API requirements may not align with how data is captured in the EHR, particularly in LTPAC care settings, and could impose unnecessary standardization burdens. The current flexibility allows developers to tailor exports to their systems' unique needs.

b. Are there specific workflow aspects that could be improved?

We *believe workflow enhancements are best handled by vendors. Market-driven improvements will better address user needs than additional regulation.*

c. Should CMS consider policy changes to support this capability's use?

The current approach, driven by vendor adaptation to market needs, remains sufficient without additional policy intervention at this stage.

4. Data Exchange

TD-12. Should CMS endorse non-CMS data sources and networks, and if so, what criteria or metrics should CMS consider?

CMS should endorse non-CMS data sources and networks in a way that strengthens—rather than fragments—national interoperability efforts. To achieve this, CMS must coordinate closely with ONC and other HHS agencies to ensure policy alignment, using TEFCA as the foundational framework for trust and technical standards.

Recommended Criteria for Endorsement:

1. *TEFCA Participation as a Baseline Requirement*
    - *Networks should actively participate in TEFCA (e.g., as QHINs or subparticipants) to ensure adherence to security, privacy, and data-sharing standards.*
    - *This creates consistency while allowing flexibility for use cases beyond TEFCA's current scope (e.g., value-based care analytics).*
2. *Interoperability and Use-Case Prioritization*
    - *Demonstrate real-world functionality for CMS priorities (e.g., care coordination, quality measurement, event notifications).*
    - *Support modern technical capabilities (e.g., FHIR APIs, bulk data access) with conformance testing.*
3. *Scalable Governance and Oversight*
    - *Align with TEFCA's governance model for transparency and patient access rights.*
    - *Leverage HHS-wide mechanisms (e.g., CMMI's experience standardizing data partnerships) to streamline vendor onboarding and monitoring.*

TD-13. What new opportunities and advancements could emerge with APIs providing access to the entirety of a patient's electronic health information (EHI)?

APIs that provide access to a patient's full electronic health information (EHI) could unlock transformative opportunities in healthcare interoperability and data-driven care. By enabling federated data lookup—where queries dynamically retrieve records from disparate sources rather than relying on centralized storage—these APIs could support real-time, comprehensive patient insights across care settings. Clinicians could access a holistic view of a patient's history, including unstructured notes, specialty-specific data, and post-acute care documentation, improving care coordination and reducing redundant tests.

Additionally, federated approaches would allow flexible data integration without mandating rigid standardization, accommodating variations in how different healthcare settings document care. Advanced analytics, AI-driven decision support, and population health tools could leverage this expanded data access, uncovering patterns that structured datasets alone might miss.

a. What are the primary obstacles to this?

The current challenge today lies in the significant effort required to map and maintain proprietary LTPAC data structures to standardized APIs like FHIR. Mandating rigid API compliance without accounting for care-setting-specific differences may also lead to unrealistic expectations for data availability, as some information simply isn't captured in certain clinical environments.

The primary obstacles to federating data versus enforcing strict adherence to standardized formats like FHIR include the complexity of mapping and maintaining proprietary LTPAC (Long-Term and Post-Acute Care) data structures to these APIs.

b. What are the primary tradeoffs between USCDI and full EHI, especially given more flexible data processing capabilities today?

The tradeoffs between USCDI (U.S. Core Data for Interoperability) and full EHI (Electronic Health Information) have diminished with advancements in flexible data processing. While USCDI provides a curated, standardized dataset for interoperability, full EHI access enables richer, context-aware decision-making by including unstructured notes, specialty-specific data, and non-standardized documentation. However, USCDI's structured approach ensures consistency, whereas full EHI may introduce variability in data quality and usability. With modern data processing capabilities, complementary solutions (such as NLP or vendor-specific enhancements) can bridge EHI gaps, allowing EHR vendors to tailor access methods without forcing a one-size-fits-all standard. This balance ensures broader data availability while maintaining usability for end-users.

TD-17. Given operational costs, what role should CMS or ASTP/ONC or both have in ensuring viability of healthcare data sharing networks, including enough supply and demand, that results in usage and outcomes?

To sustain the operational viability of healthcare data sharing networks like TEFCA, CMS and ASTP/ONC must take a proactive role in driving both supply and demand through coordinated policy and incentives. As the only federally endorsed national interoperability framework, TEFCA provides a critical foundation—but its success depends on federal leadership and financial alignment.

CMS should lead by example by joining TEFCA (as IHS has done) and integrating it into core programs (e.g., quality reporting, fraud detection). CMS should also link reimbursement incentives (e.g., Medicare/Medicaid payment adjustments) to TEFCA participation, ensuring providers, payers, and states prioritize adoption rather than creating fragmented alternatives. By embedding TEFCA into value-based care and administrative workflows, CMS can transform it from an optional framework into the default infrastructure for interoperable data exchange, ensuring both usage and measurable outcomes.

In addition, as CMS and ASTP/ONC consider this topic, they should keep in mind the wide range of existing networks including those that support public health-related use cases. For instance, Audacious Inquiry, a PointClickCare Company, manages the technical and program aspects of the Centers for Disease Control and Prevention (CDC) Immunization (IZ) Gateway which enables the exchange of data between Immunization Information Systems (IISs) and multi-jurisdictional vaccine providers ensuring more complete records of patient immunization history. The IZ Gateway streamlines immunization data exchange through a centralized policy and technology infrastructure. The IZ Gateway supports four use cases: 1) IIS-to-IIS, 2) provider-to-IIS, 3) consumer access, 4) IIS reporting to CDC. The IZ Gateway reduces the burden on providers and IIS to exchange data for a more complete view of patient and population immunization rates.

As of June 2025, the IZ Gateway supports 120 bi-directional Provider-IIS connections, 370 IIS-IIS connections, and 28 jurisdictions submitting data to CDC. Some providers currently using the IZ Gateway include the Veterans Administration, the Department of Defense, the Division of Global Migration Health,

Fond du Lac, DocStation, AZOVA, and Docket. The IZ Gateway routes between 4 and 13 million messages per month

The IZ Gateway is a successful example of a public health network that has buy-in from STLTs and CDC. When designing future policies, it will be important to consider the implications for use cases like public health that often do not have market based solutions.

5. Compliance

TD-18. Information blocking:

a. Could you, as a technology vendor, provide examples for the types of practices you have experienced that may constitute information blocking. Please include both situations of non-responsiveness as well as situations that may cause a failure or unusable response?

From a technology vendor perspective, the framework of information blocking is well defined however the ambiguity that likely continues to cause consternation from stakeholders is a lack of definition around the exceptions. While examples are provided in the exceptions, the different types of data sharing between different types of entities should be considered when further deliberating how the exceptions should be applied. As an example, there should be a level of deference provided to certified EHR providers who willingly participate in CMS requirements as stewards of patients sensitive data.

b. What additional policies could ASTP/ONC and CMS implement to further discourage healthcare providers from engaging in information blocking practices?

A key policy that CMS should pursue on behalf of all stakeholders is preempting states from implementing their own regulations that create redundancies or conflicts in policy especially as it relates to the exceptions. Having fragmented state level information blocking regulations creates significant burden and developer costs for technology vendors which inevitably creates inefficiencies in the system. Further, states being able to alter exceptions or dictate their own definitions of information blocking and maintaining state level enforcement authority will inevitably lead to conflict in the rules of the road. These conflicts can also lead to private right of action claims being invoked at the state level which will have a chilling effect for technology developers. All policy development should be in support of establishing a national framework with a coherent and cohesive set of standards to follow.

Value Based Care Organizations

1. Digital Health Adoption

VB-1. What incentives could encourage APMs such as accountable care organizations (ACOs) or participants in Medicare Shared Savings Program (MSSP) to leverage digital health management and care

navigation products more often and more effectively with their patients? What are the current obstacles preventing broader digital product adoption for patients in ACOs?

ACOs and MSSP participants could significantly benefit from digital health tools, but adoption lags due to financial and operational hurdles. To bridge this gap, we need practical solutions that address both incentives and implementation barriers.

Direct CMS funding for digital tools—like care coordination platforms and remote monitoring systems—would jumpstart adoption, especially in post-acute and home-based care. Creating dedicated reimbursement codes for digital care navigation and offering bonus shared savings for proven tech use would further motivate ACOs.

Fragmented data remains a major roadblock. ACOs need real-time insights across care settings, which requires seamless data exchange. Mandating FHIR-based API integration for Medicare/Medicaid providers would ensure hospitals, SNFs, and home health agencies can share critical patient information effortlessly.

Digital tools must reduce, not add, administrative work. Integrating care navigation and predictive analytics directly into existing EHR systems would make adoption smoother for busy care teams. A dedicated "Digital ACO" track within MSSP, with higher shared savings for tech-savvy organizations, could accelerate progress. Pairing financial rewards with interoperability standards and user-friendly design will help digital health tools deliver on their promise in value-based care.

VB-4. What are the essential data types needed for successful participation in value- based care arrangements?

For value-based care (VBC) to succeed, data must flow seamlessly across settings while remaining actionable and relevant. Below are the essential data types, refined to balance comprehensiveness with practicality:

For Skilled Nursing Facilities (SNFs):

- *Real-time ADT notifications (to enable rapid care coordination)*
- *Therapy intensity/duration (type, minutes, progress toward goals)*
- *Standardized change-of-condition alerts (e.g. early deterioration signals)*
- *Functional & cognitive trends (MDS 3.0, PDPM classifiers, mobility benchmarks)*
- *Hospitalization avoidance metrics (successful interventions, avoidable readmission flags)*
- *Pre-discharge home assessments (environmental risks, caregiver readiness)*

For Assisted Living Facilities (ALFs):

- *Standardized documentation (similar to MDS but scaled for ALF workflows)*
- *Medication adherence & high-risk drug flags (to prevent adverse events)*
- *Incident tracking (falls, elopements, infections)*

Recommendation: Pilot an ALF-specific assessment tool in states with 1115 waivers to test feasibility.

For LTC Pharmacies:

- *Medication regimen complexity scores (to flag high-risk residents)*
- *Pharmacist-driven insights (deprescribing opportunities, therapy recommendations)*
- *Real-time interaction alerts (drug-disease, drug-drug conflicts)*
- *Transition-of-care reconciliation logs (to prevent post-discharge errors)*

2. Compliance and Certification

VB-7. How can technology requirements for APMs, established through CEHRT or other pathways, reduce complexity while preserving necessary flexibility?

To balance simplicity and flexibility in APM technology requirements, CMS should adopt a tiered approach to CEHRT that recognizes the unique needs of different care settings. For SNFs, implementing a streamlined variation focused on core value-based care functions - clinical alerts, provider attribution, and quality reporting - would reduce implementation burdens while maintaining essential capabilities. Simultaneously, CMS should expand interoperability standards to incorporate long-term care pharmacy systems as equal partners in the digital ecosystem. These platforms contain vital medication insights that could significantly reduce hospitalizations if properly integrated with primary documentation systems and appropriately compensated.

By tailoring requirements to setting-specific workflows while breaking down critical data silos, we can create a more functional, less burdensome digital infrastructure for value-based care across the continuum. This approach would preserve necessary flexibility while ensuring all providers can meaningfully participate in APMs without facing unrealistic technology demands.

3. Technical Standards

VB-13. What improvements to existing criteria and standards would better support value-based care capabilities while reducing provider burden?

To better support value-based care while reducing burden, CMS should modernize interoperability requirements to reflect real-world care transitions. Three key improvements would help: First, accelerate API adoption in post-acute settings by creating simplified implementation guides tailored for SNFs/ALFs that maintain core data fluidity requirements without demanding hospital-grade infrastructure. Second, align TEFCA or trusted exchange implementation with VBC payment structures to ensure quality measures and attribution data flow seamlessly between providers and payers. Third, recognize long-term care pharmacy systems as essential clinical partners by including them in interoperability standards and quality reporting frameworks. These targeted refinements would maintain rigorous data exchange standards while respecting the operational realities of different care settings - ultimately strengthening value-based care capabilities without creating unnecessary technological burdens. The goal should be

creating "interoperability with purpose" - focused on exchanging the right data at the right time to support care coordination and quality improvement.